Cairnly

Cairnly sells supply forecasting and multi-distributor ordering software to independent US dental practices, priced per location per month.

Cairnly targets a real gap where independent practices default to distributor reps, but five paying customers and n=5 savings data leave the wedge unproven.

Evidence for the thesis: Partially supported. Interviews credibly document the workflow, the rep-default dynamic and multi-distributor buying. Evidence of winning and retaining customers against incumbents is thin: five paying customers, internal n=5 savings data, and nothing on distributor response.

How customers solve it today

In an independent practice, the office manager runs supply ordering on top of the front desk job. They walk the closet, update a spreadsheet, and usually accept the monthly re-order the distributor rep suggests. Gaps get filled from Amazon Business or a second distributor at retail prices, and invoices are matched to deliveries by hand at month end. Cairnly reports this takes about 5 hours a week. In its 22 interviews, no practice tracked usage per procedure, and 17 bought from two or more distributors.

Source: Deck s.2, 3; Cairnly customer interviews p.1, 2

Why those approaches fail

The failure is structural. The person who places the order has no time to check it, so the vendor effectively sets the order. This likely favors the distributor's catalog and margins over the practice's costs, though that is an inference. Usage knowledge lives in one person and one spreadsheet, so ordering stops when that person is away. Interviewed practices reported about three stockouts of a critical item per quarter, and these cost them rescheduled patients, not only money. Practices split purchases across distributors, but each portal shows only its own catalog and prices, so nobody sees the whole picture. Meanwhile, DSOs centralize purchasing and win better prices, which widens the cost gap for independents.

Source: Deck s.2, 3, 5, 6; Cairnly customer interviews p.1, 2

The alternatives a customer weighs

Alternative	Who uses it, and how	Where it breaks	Source
Spreadsheet plus the distributor rep's suggested order status quo	Office manager counts stock by hand; rep proposes monthly re-order that usually gets accepted unchecked.	No usage forecast, depends on one person, and the vendor effectively decides the order.	Deck s.3, 6; Cairnly customer interviews p.1, 2
Distributor ordering portals incumbent	Free with the distributor account; practices order online from their main distributor.	Locked to one distributor's catalog and prices, while most practices buy from several.	Deck s.5, 6; Cairnly customer interviews p.2
Group purchasing organizations adjacent tool	Members get negotiated discounts on supplies.	Address price only; no ordering workflow, forecasting or stockout prevention.	Deck s.6
Horizontal inventory apps adjacent tool	Generic stock tracking tools some practices use to count inventory.	No link to the procedure schedule, so they cannot forecast usage.	Deck s.6

Market structure

Supply distribution is concentrated in two national distributors, and each controls its own free ordering portal and a rep relationship that shapes what gets ordered. The buyers are fragmented. The deck estimates about 70% of roughly 135,000 US practices are independent, and each has no purchasing staff. DSOs are absorbing practices and centralizing purchasing, which squeezes independents on price. Purchasing groups help on price but not on workflow. This suggests the incumbents have the distribution and little incentive to build a neutral tool that compares across distributors.

Source: Deck s.5, 6

Where the wedge emerges

The proposed entry point is the independent practice that already buys from two or more distributors and has no forecast. That was 17 of 22 interviewees. Cairnly reads scheduled procedures from the practice management system to forecast usage, then routes each order to the best contracted price across distributors. No single distributor's portal offers that cross-distributor view. The pitch is fewer stockouts plus lower spend for a buyer that cannot match DSO purchasing scale. The founder's DSO operations background fits this. For the wedge to widen, several things must be true. Distributors must keep contracted pricing and ordering accessible to a third party rather than restrict it. Practice management integrations must work across the systems independents use. The 11% savings must hold beyond five internal pilots and clearly exceed the $249 monthly fee. The deck does not yet show any of these, and slide 8 was unreadable.

Source: Deck s.4, 5, 7, 8, 9; Cairnly customer interviews p.2

What the materials do not show

- Independent verification of the 11% savings beyond five internal pilots
- How distributors respond to third-party ordering and access to contracted pricing
- Retention and conversion from 14 pilots to paying customers
- Which practice management systems Cairnly integrates with, and their coverage

Questions for the founder

- Have either national distributor restricted or supported your ordering access, and what happens if they cut it off?
- Why have only 5 of 14 pilot practices converted to paid, and what did non-payers say?
- How do you sell around the rep relationship that currently sets most practices' orders?

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